

博 士 学 位 论 文

动态记忆应力的生物力学研究
及临床意义探讨

许 硕 贵

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第 二 军 医 大 学

DOCTOR'S DISSERTATION:

Shuogui Xu*, Biomechanical Study and clinical Significance Discussion of Dynamic Memorial Stress, Second Military Medical University, 2001.05.

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序号	专利名称	授权号	发明人	授权时间	专利类别
1	一种取薄片系统及其方法	201518009794.3	许硕贵; 何滨	2019年03月15日	国防发明专利
2	C-ARM X-RAY APPARATUS WITH SUR	1046332682	Bin He; Shigui Yan; Shuogui Xu; Liping Shen	2019年11月05日	国际专利
3	FRACTURE DEDUCTION SYSTEM	11083496B2	Bin He; Shuogui Xu; Rui Tong; Yu Wu	2021年08月10日	国际专利
4	靶点路径的确定方法、装置和系统、可读存储介质	201910161263.3	沈丽萍; 许硕贵; 何滨; 章睿; 孙伟伟	2020年12月08日	发明专利
5	带有手术定位和线性导航功能的C形臂X光机	201510196237.6	何滨, 严世贵, 许硕贵, 沈丽萍	2016年10月05日	发明专利
6	基于生物打印技术的细胞-生物支架复合物的制备方法	201310250953.9	谢杨, 孙晓飞, 章云童, 韩信, 毛宁方, 杨永信, 倪海健, 许硕贵, 夏琰	2014年07月09日	发明专利
7	基于无创式实时手术定位导航设备的骨科手术机器人	201610047353.6	何滨, 沈丽萍, 许硕贵, 章睿	2017年09月08日	发明专利
8	具有电响应特性的形状记忆材料及制备方法	200410073393.5	朱光明; 秦瑞丰; 许硕贵;	2006年08月09日	发明专利
9	无创式实时手术定位3D导航设备	201610659818.3	许硕贵, 何滨, 沈丽萍	2019年01月04日	发明专利
10	无创式实时手术定位导航设备	201310515922.1	何滨, 许硕贵	2015年09月09日	发明专利
11	一种动物骨髓损伤造模装置	201410554225.1	谢杨, 李超, 孙晓飞, 金光辉, 章云童, 许硕贵, 杨国士, 张黎雯, 夏琰	2016年01月06日	发明专利
12	一种纵向梯度孔结构的纳米人工骨支架及其制备方法	201110287964.5	许硕贵, 周斌, 夏琰, 唐骏, 杜艾, 卢春阔, 张猛	2014年12月10日	发明专利
13	一种类似天然骨结构的纳米人工骨支架及其制备方法	201310432628.4	许硕贵, 王红英, 夏琰, 张艳艳, 周潘宇, 程晓松	2017年04月12日	发明专利
14	一种生物可吸收捆绑带及其形状记忆锁扣	200610023175.X	许硕贵; 朱光明; 张春才;	2007年12月19日	发明专利
15	一种用于骨折治疗的智能个性化外骨骼装置	201610914541.4	许硕贵; 何滨;	2017年09月08日	发明专利
16	尺桡骨形状记忆接骨器	00216128.1	张春才, 许硕贵, 苟三怀, 冯永辉, 张德胜	2001年02月28日	实用新型专利
17	大鼠腹部注射固定装置	201720230380.7	袁野; 李佳浓; 周潘宇; 许硕贵; 姜里强; 张洪跃; 吴江红;	2018年05月22日	实用新型专利
18	带有手术定位和线性导航功能的C形臂X光机	201520250205.5	何滨, 严世贵, 许硕贵, 沈丽萍	2015年09月09日	实用新型专利
19	防冷凝的呼吸软管及呼吸湿化设备	201920424104.3	姜立国; 许硕贵;	2020年03月17日	实用新型专利
20	肱骨远端镍钛记忆合金接骨器	02217427.3	张春才; 张德胜; 苟三怀; 张雪松; 李亚茂; 曲乐丰; 王芝芳; 许硕贵;	2003年05月07日	实用新型专利
21	股骨颈骨折空心螺钉导向装置	201820569864.9	许硕贵; 吴江红; 何滨; 周潘宇; 袁野; 陈汉清;	2019年04月12日	实用新型专利
22	骨骼复位系统	201721567092.7	何滨; 许硕贵; 章睿; 吴雨;	2019年06月04日	实用新型专利
23	骨骼复位装置	201721568294.3	何滨; 许硕贵; 方华磊; 王婧;	2019年02月05日	实用新型专利
24	骨折复位及维持装置	201821279863.7	吴江红; 吴雨斐; 许硕贵; 周潘宇; 夏伊玲; 吴琦; 夏慧娟; 张洪跃;	2019年10月11日	实用新型专利
25	基于C臂X光机的激光定位仪及手术导航系统	201920068139.8	许硕贵; 吴江红; 何滨; 周潘宇; 夏德明; 李磊; 汪洋;	2020年01月17日	实用新型专利
26	基于无创式实时手术定位导航设备的骨科手术机器人	201620068764.9	何滨, 沈丽萍, 许硕贵, 章睿	2016年08月17日	实用新型专利
27	肩锁关节弹性镍钛记忆合金固定器	02217424.9	张德胜; 许硕贵;	2003年05月07日	实用新型专利
28	具有升降功能的旋转式激光定位导航标定装置	201720852596.7	袁野; 周潘宇; 许硕贵; 吴江红; 姜里强; 张洪跃; 白思嘉; 徐金山;	2019年02月15日	实用新型专利
29	聚醚醚撑开器	02215441.8	张德胜; 张春才; 许硕贵; 梁洁; 孙兰萍; 王芝芳; 苟三怀;	2002年12月11日	实用新型专利
30	可生物降解的血管吻合环/轮及其专用手术器械	201720605628.3	许硕贵; 汤燕; 肖亮; 严新道; 王攀峰; 周潘宇;	2018年11月06日	实用新型专利
31	可吸收粗隆截骨固定组合器	201020525734.9	许硕贵, 王吉广, 方永刚, 张春才	2011年05月04日	实用新型专利
32	可吸收弓形记忆接骨器	201020525762.0	许硕贵, 刘德鼎, 徐斌, 王吉广, 张春才	2011年06月29日	实用新型专利
33	可吸收髓内钉柱解髓记忆固定器	201020525783.2	许硕贵, 徐斌, 刘德鼎, 张春才	2011年09月14日	实用新型专利
34	可吸收髓内钉柱解髓记忆固定器	201020525765.4	许硕贵, 方永刚, 卢春阔, 张春才, 朱光明	2011年06月01日	实用新型专利
35	可吸收三维解剖型髓内记忆内固定系统	201020525732.X	许硕贵, 吴亚乐, 华夏, 张春才	2011年06月01日	实用新型专利
36	实验用老鼠固定装置	201720230959.3	袁野; 周潘宇; 许硕贵; 李佳浓; 姜里强; 张洪跃; 吴江红;	2018年05月22日	实用新型专利
37	无创式实时手术定位3D导航设备	201620871979.4	许硕贵, 何滨, 沈丽萍	2017年06月27日	实用新型专利
38	下肢腓骨内固定镍钛合金记忆钩	02217422.2	张德胜; 张雪松; 许硕贵; 孙兰萍;	2003年07月02日	实用新型专利
39	一种半导体降温马甲	201921807420.5	张馨丹; 常文军; 李磊; 许硕贵; 张凡; 袁浩;	2020年08月14日	实用新型专利
40	一种尺骨鹰嘴形状记忆内固定器	200820057266.X	王森林, 许硕贵, 张春才	2009年02月04日	实用新型专利
41	一种带磁性的氧气筒安全警示牌	201620655794.X	胡敏, 张玲, 许硕贵, 王群	2017年03月08日	实用新型专利
42	一种带有缝合标记线的抗菌手术贴膜	201821279876.4	吴江红; 吴琦; 许硕贵; 周潘宇; 吴雨斐; 夏伊玲; 章云童; 郝淑辰;	2019年10月08日	实用新型专利

43	一种弹力绷带	201821152772.7	周潘宇 ; 夏德萌 ; 许硕贵 ; 袁野 ; 王晋人 ; 熊文娟 ; 孙大鹏 ;	2019年07月05日	实用新型专利
44	一种骨针折弯组件	201720389678.2	周潘宇 ; 吴江红 ; 许硕贵 ; 姜里强 ; 袁野 ; 张洪跃 ; 夏琰 ; 王欢 ;	2018年10月12日	实用新型专利
45	一种急救止血绷带	201821154448.9	周潘宇 ; 夏德萌 ; 许硕贵 ; 吴江红 ; 王晋人 ; 熊文娟 ; 孙大鹏 ;	2019年09月24日	实用新型专利
46	一种既可降温又可供饮水的双功能水冷降温马甲	201921807453.X	李磊 ; 常文军 ; 张馨丹 ; 许硕贵 ; 张凡 ; 袁浩 ;	2020年08月14日	实用新型专利
47	一种具有气囊结构的注射针	201822008755.2	周潘宇 ; 夏德萌 ; 许硕贵 ; 倪喆鑫 ; 李磊 ; 王晋人 ; 汪洋 ; 王硕 ;	2019年09月24日	实用新型专利
48	一种髓前上棘复位内固定器	201320070559.2	吴亚乐, 许硕贵, 王众,	2013年09月11日	实用新型专利
49	一种全地形担架	201420603360.6	谢杨, 章云童, 许硕贵, 孙晓飞, 马兵, 李超, 董微, 金光辉, 夏斌, 张世杰	2015年01月28日	实用新型专利
50	一种桡、尺动脉压迫止血器	201720418915.3	吴江红 ; 周潘宇 ; 许硕贵 ; 朱嘉琦 ; 袁野 ; 姜里强 ; 张洪跃 ; 夏琰 ;	2018年10月12日	实用新型专利
51	一种深度及暴露范围可调的自动手术拉钩	201821283529.9	吴江红 ; 吴疏 ; 许硕贵 ; 周潘宇 ; 吴雨斐 ; 夏伊玲 ; 章云童 ; 夏琰 ;	2019年10月11日	实用新型专利
52	一种适应复杂环境的多功能多用途便携式担架	202021007934.5	夏德萌 ; 桂莉 ; 范启顺 ; 许硕贵 ; 王晋人 ; 周潘宇 ; 王毅欣 ; 黄燕	2021年06月08日	实用新型专利
53	一种适用于指、掌骨骨折固定的钢板内固定装置	201220262946.1	谢杨, 李明, 陈自强, 许硕贵, 张春才, 孙晓飞,	2012年12月05日	实用新型专利
54	一种系带改进急救生衣	202021006206.2	夏德萌 ; 许硕贵 ; 郑丽雪 ; 王晋人 ; 周潘宇 ; 欧天乐 ; 范启顺	2021年02月26日	实用新型专利
55	一种腰椎固定带	201620677951.7	王立芬, 张玲娟, 席惠君, 张玲, 许硕贵,	2017年04月12日	实用新型专利
56	一种医用钢针	201621140963.2	许硕贵 ; 何滨 ;	2017年09月08日	实用新型专利
57	一种医用开口器	201520867084.9	徐立, 胡敏, 张玲, 许硕贵, 张笑平, 祁智, 张敏伶	2016年05月04日	实用新型专利
58	一种用于骨折修复的骨骼复位机构	201621141453.7	许硕贵 ; 何滨 ;	2017年09月26日	实用新型专利
59	一种用于骨折治疗的智能个性化外骨骼装置	201621141323.3	许硕贵, 何滨	2017年10月03日	实用新型专利
60	一种止血注射装置	201822009961.5	夏德萌 ; 周潘宇 ; 许硕贵 ; 孔凯文 ; 李磊 ; 王晋人 ; 张洪跃 ; 汪洋 ;	2019年09月24日	实用新型专利
61	一种止血装置	201822008698.8	周潘宇 ; 夏德萌 ; 汪洋 ; 许硕贵 ; 章云童 ; 李磊 ; 王晋人 ; 张洪跃 ;	2019年09月24日	实用新型专利
62	一种注射装置	201822011833.4	夏德萌 ; 周潘宇 ; 许硕贵 ; 张洪跃 ; 夏琰 ; 李磊 ; 王晋人 ; 汪洋 ;	2019年12月17日	实用新型专利
63	用于防护用具的密封装置和防护设备	202020162585.8	杨峰 ; 许硕贵 ; 盛春泉 ; 王丹 ; 陆斌 ; 邹晨 ; 徐丽霞 ; 王治 ; 沈素萍	2021年04月27日	实用新型专利
64	警示牌 (带磁性的氧气筒安全警示牌)	201630284702.7	胡敏, 张玲, 许硕贵, 王群,	2016年12月14日	外观设计
65	密封贴	202030600245.4	杨峰 ; 许硕贵 ; 盛春泉 ; 王丹 ; 陆斌 ; 邹晨 ; 徐丽霞 ; 王治 ; 沈素萍	2020年12月15日	外观设计
66	桡下骨折记忆加压接骨器	01344219.8	张春才, 徐卫东, 许硕贵,	2002年05月08日	外观设计
67	弓齿形状记忆接骨器 (III型)	01344220.1	张春才, 许硕贵, 苏佳灿,	2002年05月08日	外观设计
68	弓齿形状记忆接骨器 (II型)	01344221.X	张春才, 许硕贵, 苏佳灿,	2002年05月08日	外观设计
69	弓齿形状记忆接骨器 (I型)	01344222.8	张春才, 许硕贵, 苏佳灿,	2002年05月08日	外观设计
70	解剖型记忆加压接骨器	01344218.X	张春才, 许硕贵, 孙满法,	2002年06月26日	外观设计
71	髓臼前柱解剖记忆固定器 (II型)	01344223.6	张春才, 许硕贵, 苏佳灿, 张雪松,	2002年05月08日	外观设计
72	髓臼前柱解剖记忆固定器 (I型)	01344224.4	张春才, 许硕贵, 苏佳灿, 张雪松,	2002年05月08日	外观设计
73	尺桡骨喙导向记忆固定器	01344217.1	张春才, 王家林, 许硕贵,	2003年02月05日	外观设计
74	尺桡骨形状记忆接骨器 (II型)	01352651.0	张春才, 许硕贵, 王家林, 吴玉堂,	2002年07月24日	外观设计
75	尺桡骨形状记忆接骨器 (I型)	01352648.0	张春才, 许硕贵, 苏佳灿, 孙满法,	2003年01月01日	外观设计
76	后白壁记忆固定器	01352649.9	张春才, 许硕贵, 苏佳灿, 孙满法,	2003年01月22日	外观设计
77	后柱臼解剖型记忆固定器	01352647.2	张春才 ; 许硕贵 ; 苏佳灿 ; 吴玉堂 ;	2002年06月26日	外观设计
78	记忆接骨器 (髓臼后柱II型)	200730146002.2	张春才 ; 许硕贵 ; 达国祖 ;	2008年03月19日	外观设计
79	接骨器 (弓齿形状记忆型)	200730145895.9	张春才 ; 许硕贵 ; 达国祖 ;	2008年04月23日	外观设计
80	接骨器 (天鹅式记忆型)	200730146005.6	张春才 ; 许硕贵 ; 达国祖 ;	2008年04月23日	外观设计
81	白后壁记忆接骨器 (I型)	200730145896.3	张春才 ; 许硕贵 ; 达国祖 ;	2008年06月25日	外观设计
82	髓臼后柱记忆固定器	01352650.2	张春才, 许硕贵, 孙满法,	2002年10月16日	外观设计
83	髓臼后柱记忆接骨器 (I型)	200730145900.6	张春才 ; 许硕贵 ; 达国祖 ;	2009年03月25日	外观设计
84	髓臼前柱记忆接骨器 (I型)	200730145898.2	张春才 ; 许硕贵 ; 达国祖 ;	2008年03月19日	外观设计
85	髓臼前柱记忆接骨器 (II型)	200730145899.7	张春才 ; 许硕贵 ; 达国祖 ;	2008年03月19日	外观设计
86	形状记忆接骨器	01352415.1	张春才, 王家林, 许硕贵, 孙满法, 吴玉堂,	2002年10月16日	外观设计

REGISTRATION CERTIFICATE FOR MEDICAL DEVICE:



Memory Fixator for Greater Trochanteric Osteotomy, Reg. No.: 3460861.



Memory Fixator for Posterior Acetabular Wall Guidance, Reg. No.: 3460862.



Memory Alloy Patellar Claw
Reg. No.: 3460482.



Serrated Memory Cerclage Ring
Fixator, Reg. No.: 3460860.



Anatomical Memory Fixator for Posterior Acetabular Column, Reg. No.: 3460863.



Anatomical Memory Fixator for Anterior Acetabular Column, Reg. No.: 3460858.



Swan Memory Fixator, Reg. No.: 3460857.

查新结论

上海长海医院委托之查新项目“髌臼骨折治疗的基础与临床研究”，经 CBMDISC (1978-2010, 5) 和 MEDLINE (1991-2010, 5) 光盘数据库检索及文献追踪：

髌臼位置深在，解剖关系复杂，髌臼骨折为高速高能量损伤的关节内骨折，易导致多种并发症的出现，延误诊治。因此髌臼骨折能否得到及时合理的诊治是确保其预后良好的关键。目前切开复位内固定已经成为移位性髌臼骨折的标准治疗方法，其治疗目标在于髌臼的解剖复位和坚强内固定，从而使早期活动成为可能。Judet-Letoumel 的髌臼三柱概念和骨折分类及手术入路，已成为髌臼骨折治疗的里程碑【1-3】。髌臼骨折的内固定多采用钢板和螺钉，其中有采用钛钢板和镍钛记忆合金钉【4-5】。

关于髌臼骨折手术后的恢复效果，国外文献6报道，对60个平均随访5年的髌臼骨折外科治疗的患者评估疗效，其解剖复位率是62%，临床结果优良率为80%。文献7报道，髌臼骨折后路入路的外科治疗长期随访（10-15年）后，优良率为80%。

该课题组在分析研究髌臼的解剖与生物力学结构基础上，提出了髌臼的三柱理论，建立了髌臼骨折浮动分类方法，并以此为基础，设计了髌臼三维记忆内固定系统，展开了系列基础研究与临床研究。临床应用解剖复位率 92.7%，功能优良率 92.69%，明显高出国外水平，为解决复杂粉碎的髌臼骨折提供了一种有效的新方法。国内外未见与该课题组相同的文献报道。

查新单位



(盖章)



2010年9月19日

备注：

本结论仅供专家参考

Basic and Clinical Research on Acetabular Fracture Treatment. Anatomical reduction rate improved from 62% to 92.7%; functional excellent rate from 80% to 92.69%, surpassing international benchmarks. Conclusion: No identical prior literature found domestically or internationally. Issued by: Changhai Hospital, Shanghai, 2010.09.19.

许硕贵教授课题组发明的系列记忆接骨器临床应用例数统计 (2014-2024)

年份	2014	2015	2016	2017	2018	2019	2020	2021	2022	2023	2024	合计
例数	56500	71333	72578	80119	81226	94097	95683	65024	78149	77995	78452	851186
终端销售额 (万元)	47963	62351	64489	72269	73564	86959	87447	57826	70746	70279	51345	745239
备注：按已推广应用到的 3225 家医院统计												



Series of Shape Memory Bone Fixators. Applied in 3,225+ hospitals globally, including Johns Hopkins University and other international institutions. Issued by: Lanzhou Ximai Memory Alloy Co., Ltd., 2025.02.06.



Allergy to Surgical Implants

Karin A. Pacheco^{1,2}

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Abstract

Surgical implants are essential elements of repair procedures to correct worn out joints, damaged spinal components, heart and vascular disease, and chronic pain. However, many of the materials that provide stability, flexibility, and durability to the implants are also immunogenic. Fortunately, allergic responses to surgical implants are infrequent. When they do occur, however, the associated pain, swelling, inflammation, and decreased range of motion can significantly impair the implant function. Given the high numbers of joint replacements performed in the developed world, allergic reactions to orthopedic implants form the largest category of allergic responses. The most important allergens in this category include nickel, cobalt, titanium, and bone cement. These allergens are also the most important in reactions to spinal surgery. Multiple cardiac and neuromuscular devices are constructed of metals and adhesives that can be sensitizing to some individuals. Implantable pulse generators, important in cardiac pacemakers, gastric stimulators, and neurostimulators, may include components made of titanium steel, titanium alloy, platinum and iridium, epoxy resins, poly methyl methacrylates, and acrylates, all of which are immunogenic in some patients. Cardiac stents and pacemakers are often made of Nitinol, a composite of nickel and titanium. Most surgical procedures are closed using skin glue, which are also capable of triggering a hypersensitive contact dermatitis. Patch testing is the gold standard to determine sensitization, and this review provides a list of standard allergens to test for different implants. The patients most appropriate for testing include (1) pre-operative joint replacement patients with a prior history of skin reactions to metal jewelry, joint signs, wound healing, metal glass lenses, artificial nails, or skin glue; (2) post-operative joint replacement failure patients needing revision without an obvious cause such as infection or mechanical incompatibility; and (3) post-operative cardiac or neurological patients with localized rash, pain, swelling, or inflammation near or over the implant.

Keywords Surgical implant · Joint replacement · Joint failure · Rash · Metal allergy · Nickel · Adhesive allergy · Methyl methacrylate · Contact dermatitis

Introduction

Surgical implants rely on a number of materials selected on the basis of their strength, stability, pliability, durability, and other physical characteristics. Unfortunately, some of these materials also have allergenic properties that can cause

immunological reactions and implant failure for a small number of patients. This review will provide background information on the market, kinds, and various uses of orthopedic joint implants, as these are the great majority of surgical implants used in the USA. They form the largest category of Medicare expenditures and increasingly represent a life-altering event for a patient. They cause both a physical and financial burden on patient, caregiver, and payer. Implant components, materials used, and repair cases will illustrate the range of orthopedic replacement and adverse outcomes reported. The materials and immune consequences of other implants, including cardiac components, pacemakers, and pacemaker implantation, will be examined. While some surgical exposure elicits reactions that allergy to orthopedic implants is a real condition, there is ample evidence in the published literature of both contact dermatitis and respiratory disease triggered by sensitization.

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ORIGINAL ARTICLE

Determining the Mechanical Properties of Super-Elastic Nitinol Bone Staples Through an Integrated Experimental and Computational Calibration Approach

Dario Carbonaro¹ · Claudio Chiavara¹ · Federico A. Bologna¹ · Alberto L. Audeolino² · Mara Tassinari¹

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Abstract

Super-elastic bone staples have emerged as a safe and effective alternative for internal fixation. Nevertheless, several biomechanical aspects of super-elastic staples are still unclear and require further exploration. Within this context, this study presents a combined experimental and computational approach to investigate the mechanical characteristics of super-elastic staples. Two commercially available staples with distinct geometry, characterized by two and four legs, respectively, were examined. Experimental four-point bending tests were conducted to evaluate staple performance in terms of generated forces. Subsequently, a finite element-based calibration procedure was developed to capture the unique super-elastic behavior of the staple materials. Finally, a virtual bending testing framework was implemented to separate the effect of geometry from that of the material characteristics on the mechanical properties of the devices, including generated force, strain distribution, and fatigue behavior. The experimental tests indicated differences in the force vs. displacement curves between staples. The material calibration procedure revealed marked differences in the super-elastic properties of the materials employed in staple 1 and staple 2. The results obtained from the virtual bending testing framework have showed that both geometric features and material characteristics had a relevant impact on the mechanical properties of the device, especially on the generated force, whereas their effect on strain distribution and fatigue behavior was comparatively less pronounced. To conclude, this study advances the biomechanical understanding of Nitinol super-elastic staples by separately investigating the impact of geometry and material characteristics on the mechanical properties of two commercially available devices.

Keywords Bone staple · Orthopedic device · Nickel-titanium · Experimental testing · Digital twin · Material identification

Introduction

Bone staples are internal fixation devices that are commonly used in the foot, hand, and ankle to provide bone alignment and stabilization after fracture or fusion procedures [1, 17, 25]. Nitinol bone staples have emerged as a safe and effective alternative for internal fixation [1, 15, 21, 31], exhibiting superior stability, reduced surgery time and increased mechanical characteristics, as compared to conventional

stainless-steel staples [27, 28]. Upon insertion into predrilled holes, Nitinol staples recover their initial shape and apply compressive forces at the deployment site, pulling fractured bones together to maintain proper alignment [18]. The shape recovery mechanism of these staples can be related either to the super-elastic or to the shape memory properties characterizing Nitinol [16, 28]. Accordingly, Nitinol staples are classified into super-elastic and body temperature activated (or heat-activated) staples. In the first type of Nitinol staples, austenite finish (A_f) transformation temperature of Nitinol (T_A) is approximately equal to or lower than room temperature. These staples must be held open by a surgical instrument before deployment. In the second type of Nitinol staples, the A_f transformation temperature of Nitinol is above room temperature. Thus, shape recovery is triggered by the thermal shape memory effect as the staple reaches body temperature or is heated to higher temperatures.

Associan Fellow Juan S. Roldán oversaw the review of this article.

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Research Article

Combustion Synthesis Porous Nitinol for Biomedical Applications

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Porous Nitinol with a three-dimensional anisotropic interconnected open pore structure has been successfully produced by the combustion synthesis (CS) of elemental Ni and Ti powders. The resulting product can be tailored to closely match the stiffness of cancellous bone to minimize stress shielding. The average elastic modulus was approximately 1 GPa for a porosity of 60 vol% and the average pore size of 800–500 μm . The low elastic modulus meets the basic demand for orthopedic bone ingrowth applications. Furthermore, porous Nitinol was composed of cubic (austenitic) and monoclinic (martensitic) NiTi compounds without the presence of Ni-rich or Ni-poor phases. The resulting product exhibits excellent corrosion resistance with breakdown potentials above 2500 mV. An *in vivo* study in cortical sites of the tibia demonstrated rapid osseointegration into the porous structure as early as two weeks and complete bone growth across the implant at six weeks. A separate *in vitro* study showed complete through-growth of bone at four months using a human interbody fusion model, substantiating the use of porous Nitinol as an implant material for applications in the spine. Porous Nitinol is thus a promising biomaterial with good biocompatibility and exceptional osseointegration performance which may enhance the healing process and promote long-term fixation, making it a strong candidate for a wide range of orthopedic implant applications.

1. Introduction

Since the early 1960s, orthopedic implants were made from three distinct material types, metals, ceramics, and polymers. Commonly used materials are cobalt base metal alloys, stainless steel, titanium, polyetheretherketone (PEEK), zirconia, and alumina ceramics, poly lactic acid (PLA), or polyglycolic acid (PGA) biodegradable materials. In spine, solid titanium has historically been used for spinal fusion implants. Since the early 2000s, PEEK was introduced and gained acceptance for its low modulus and radiolucent properties [1, 2], however, recent studies have shown that PEEK does not integrate well with surrounding bone and may form a fibrous connective interface. Due to its hydrophobic and bioinert properties, the spine industry is undergoing a paradigm shift, moving back to metals to improve osseointegration at the bone-to-implant interface and to increase long-term implant stability [3].

Nitinol has widely been used for decades in cardiovascular, neurological and orthopedic devices. In its wrought state, Nitinol exhibits a modulus of elasticity of between 40–75 GPa depending upon its transformation temperature and crystallographic texture. In contrast, wrought stainless steel and titanium exhibit moduli of up to 210 and 110 GPa, respectively [4]. While the elastic modulus of cortical bone is between 17–20 GPa, that of cancellous is approximately 1.5 GPa [5–7]. The implantation of solid titanium or stainless-steel devices caused stress shielding due to the mismatch of stiffness between the implant and the surrounding bone. “Stress shielding” refers to the stress reduction and loss of bone density that results when implants remove the stresses normally experienced by bone. Stress shielding is minimized by using implants that exhibit stiffness similar to the surrounding bone. Stiffness, in turn, is influenced by both the design and by the elastic modulus of the implant material. The development of biocompatible materials that closely





国家科学技术进步奖 证 书

为表彰国家科学技术进步奖获得者，
特颁发此证书。

项目名称：动态记忆应力促进战创伤骨愈合
的关键技术与临床转化

奖励等级：二等

获 奖 者：许硕贵



2016年12月21日

证书号：2016-J-24400-2-02-R01

China National Science and Technology Progress Award

*Title: Key Technology Research and Clinical Translation of Dynamic Memory Stress for
Promoting War Trauma Bone Healing*

报告编号: 2020101C0701102

科技查新报告

项目名称: 手术激光定位导航系统

委托 人: 中国人民解放军海军军医大学第一附属医院

上海长征医院

委托日期: 二〇二〇年四月二十八日

查新机构: 中国科学院上海科技查新咨询中心

查新完成日期: 二〇二〇年五月十一日

中华人民共和国科学技术部

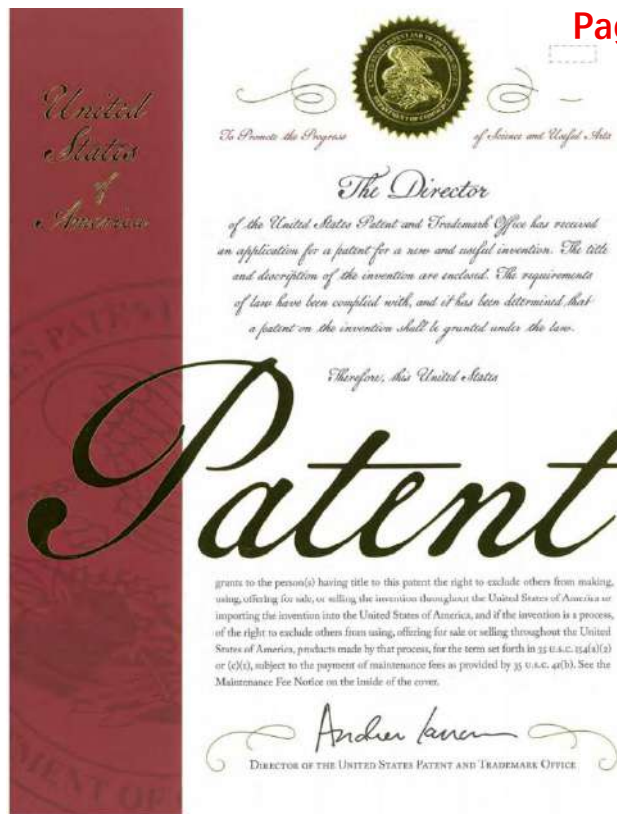
二〇一六年制

查新项目名称	中文: 手术激光定位导航系统		
	英文: Surgical laser positioning and navigation system		
来源	中国科学院上海科技查新咨询中心		
通信地址	上海市岳阳路 319 号 21 号楼 C 座四楼	邮编	200031
查新机构负责人	江洪波	联系人	刘 剑
电 话	021-54922020	传 真	021-54922034
电子邮箱	shn@sjl.cn	网 址	http://www.shn.cn
一、查新目的			
成果			
二、查新项目的科学技术要点			
创新点: 项目组(上海长征医院和杭州以达医疗科技有限公司)发明了基于 C 型臂 X 光机(C 臂机)的术中智能定位的手术激光定位导航系统。项目组根据 C 臂机成像原理,首次构建了 C 臂机透视成像“数学模型”,首次提出了“体内目标成像射线空间可视化”的全新手术导航模式。通过设计“激光穿透成像机构、基座垂直加配打靶准孔特制加工法”等硬件技术和“定位模型算法、图像识别、畸变校正等”等软件技术,并经过上千次实验,最终发明了手术激光定位导航系统。并实现了体内靶点的毫米级定位精度和手术路径智能化可视二维导航。			
技术指标: 定位精度高,误差小于 1.5mm;定位迅速,单次定位时间小于 5s;有效减少了患者和手术医护人员辐射暴露。			
三、查新点与查新要求			
查新点: 手术激光定位导航系统 1. 首次构建了 C 臂机透视成像“数学模型”(或称“同心圆定位原理”)。 2. 首次提出了“体内目标成像射线空间可视化模式”(英文称“X-ray Trajectory Visualization during Target Imaging”)的全新手术导航模式。 3. 首次设计“激光穿透成像机构、基座垂直加配打靶准孔特制加工法”等硬件技术和“定位模型算法、图像识别、畸变校正等”等软件技术,实现了定位误差小于 1.5mm,单次定位时间小于 5s,从而提高了手术操作精度,减少了手术创伤,缩短了手术时间及降低了患者和手术医护人员的辐射暴露。			
查新要求: 国内外有文献或专利文献与研究报道,并在对比分析及新颖性判断。			

Surgical Laser Positioning and Navigation System. Positioning accuracy $\leq 0.1\text{mm}$ (clinical overall accuracy $\leq 1.0\text{mm}$); single positioning time $< 5\text{s}$; effectively reduces radiation exposure for patients and surgical staff. Conclusion: No technically identical prior literature found domestically or internationally. Issued by: Shanghai Science and Technology Information Consulting Center, Chinese Academy of Sciences, 2020.05.11.

序号	发明名称	专利类别	专利号	所有发明人
1	无创式实时手术定位导航设备	发明专利	ZL201310515922.1	何滨、 许硕贵
2	带有手术定位和线性导航功能的C形臂X光机	发明专利	ZL201510196237.6	何滨、严世贵、 许硕贵 、沈丽萍
3	基于无创式实时手术定位导航设备的骨科手术机器人	发明专利	ZL201610047353.6	何滨、沈丽萍、 许硕贵 、童春
4	无创式实时手术定位3D导航设备	发明专利	ZL201610659818.3	许硕贵 、何滨、沈丽萍
5	靶点路径的确定方法、装置和系统、可读存储介质	发明专利	ZL201910161263.3	沈丽萍、 许硕贵 、何滨、童春、孙伟伟
6	一种取弹片系统及其方法	国防发明专利	ZL201518009794.3	许硕贵 、何滨
7	C-ARM X-RAY APPARATUS WITH SURGICAL POSITIONING AND LINEAR NAVIGATION FUNCTION	美国发明专利	US10463326B2	何滨、严世贵、 许硕贵 、沈丽萍
8	带有手术定位和线性导航功能的C形臂X光机	实用新型专利	ZL201520250205.5	何滨、严世贵、 许硕贵 、沈丽萍
9	基于无创式实时手术定位导航设备的骨科手术机器人	实用新型专利	ZL201620068764.9	何滨、沈丽萍、 许硕贵 、童春
10	无创式实时手术定位3D导航设备	实用新型专利	ZL201620871979.4	许硕贵 、何滨、沈丽萍
11	具有升降功能的旋转式激光定位导航标定装置	实用新型专利	ZL201720852596.7	袁野、周潘宇、 许硕贵 、吴江红、姜里强、张洪跃、白思嘉、徐金山
12	基于C臂X光机的激光定位仪及手术导航系统	实用新型专利	ZL201920068139.8	许硕贵 、吴江红、何滨、周潘宇、夏德萌、李磊、汪洋

1. [Shuogui Xu](#), Jianghong Wu, Bin He, Panyu Zhou, Demeng Xia, Lei Li and Yang Wang. Laser Positioning Device and Surgical Navigation System Based on C-arm X-ray Machine. Patent No.: ZL201920068139.8, 2020.01.17.
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11. Bin He, [Shuogui Xu](#)*. Non-invasive Real-time Surgical Positioning and Navigation Device. Patent No.: ZL201310515922.1, 2015.09.09.
12. Bin He, Shigui Yan, [Shuogui Xu](#)* and Liping Shen. C-arm X-ray Apparatus With Surgical Positioning and Linear Navigation Function. Patent No.: ZL201520250205.5, 2015.09.09.



(12) **United States Patent**
He et al.

(10) **Patent No.:** **US 10,463,326 B2**
(45) **Date of Patent:** **Nov. 5, 2019**

(54) **C-ARM X-RAY APPARATUS WITH SURGICAL POSITIONING AND LINEAR NAVIGATION FUNCTION**

(71) **Applicant:** **HANGZHOU SANTAN MEDICAL TECHNOLOGY CO., LTD.** Zhejiang (CN)

(72) **Inventors:** **Bin He, Zhejiang (CN); Shigui Yan, Zhejiang (CN); Shouqi Xu, Zhejiang (CN); Liping Shen, Zhejiang (CN)**

(73) **Assignee:** **HANGZHOU SANTAN MEDICAL TECHNOLOGY CO., LTD.** Zhejiang (CN)

(*) **Notice:** Subject to any disclaimer, the term of this patent is extended or adjusted under 35 U.S.C. 154(b) by 195 days.

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(86) **PCT No.:** **PCT/CN2015/080233**
§ 371 (c)(1),
(2) **Date:** **Sep. 21, 2017**

(87) **PCT Pub. No.:** **WO2016/169099**
PCT Pub. Date: **Oct. 27, 2016**

(65) **Prior Publication Data**
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(30) **Foreign Application Priority Data**
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(51) **Int. Cl.**
A61B 6/12 (2006.01)
A61B 6/00 (2006.01)
(Continued)

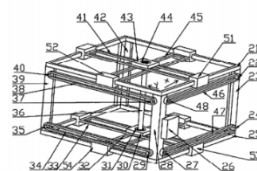
(52) **U.S. CL.**
CPC — **A61B 6/4441** (2013.01), **A61B 6/4405** (2013.01), **A61B 96/37** (2016.02);
(Continued)

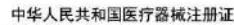
(58) **Field of Classification Search**
CPC — **A61B 6/4441**; **A61B 6/4405**; **A61B 6/00**; **A61B 6/12**; **A61B 96/37**
See application file for complete search history.

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(Continued)

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*International Search Report (Form PCT/ISA/2107), dated Aug. 26, 2015, with English translation thereof, pp. 1-4.
Primary Examiner — David P. Porta
Assistant Examiner — Shan Leo
(74) **Attorney, Agent, or Firm** — KIPRINET

(57) **ABSTRACT**
A C-arm X-ray apparatus with a surgical positioning and linear navigation function is provided. The apparatus includes a C-arm, an image enhancer, an X-ray tube, a support frame, a trolley, a monitor and a central controller. The image enhancer is disposed at one end of the C-arm, and the X-ray tube is disposed at the other end of the C-arm which is mounted on the support frame and capable of rotatingly sliding along the support frame, the support frame is mounted on the trolley and capable of sliding horizontally along the trolley, and the monitor and the central controller are electrically connected with the image enhancer and the X-ray tube, where a positioning module is (Continued)







**China Association of Inventions Entrepreneurship Award
Innovation Award**

*Title: Surgical Laser Positioning Navigation New Equipment, New Technology
and Clinical Application*

获奖证明

兹证明以下作品，参加中国国际大学生创新大赛（2025），并获得全国总决赛金奖。

作品名称：Integrated C-arm X-ray and Laser Technology for High-Precision Real-Time Surgical Positioning and Navigation

参赛学生：Shalong Xiong、Sebastian Gruber、Jinbo Ren、Hans Ewigmann、Yuanhao Wei、Minghao He、Rong Cha

指导教师：董雪、许硕贵、张晔

特此说明。





1. Shuogui Xu*, Feng Zhang. A Wearable In-Ear Core Body Thermometer. Patent No.: ZL 202221652946.2, 2022.10.21.



**Shanghai Emerging Science and Technology Collaborative Innovation
Competition Award**

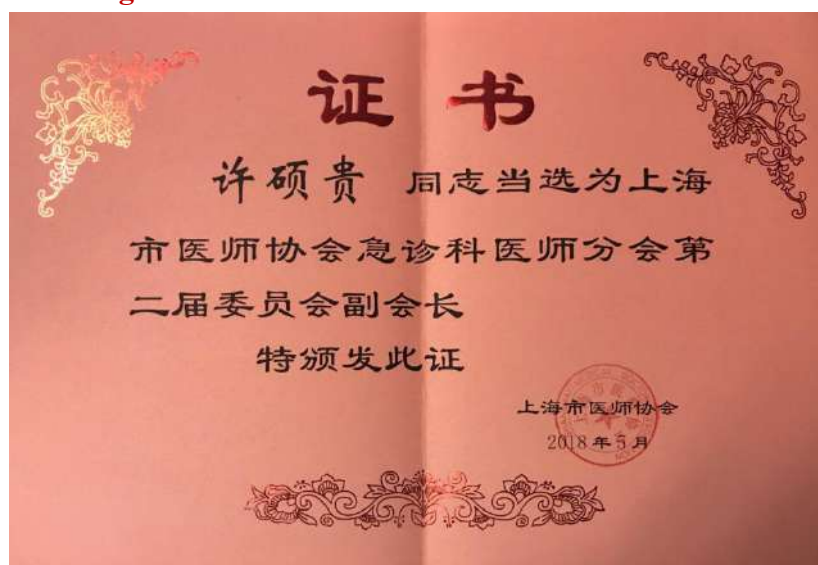
Title: Ear Canal Implantable Core Body Temperature Monitor



Vice President, Emergency Physicians Branch, Chinese Medical Doctor Association



Vice Chair, Emergency Medicine Subspecialty Branch, Shanghai Medical Association.



Vice President, Emergency Physicians Branch, Shanghai Medical Doctor Association.



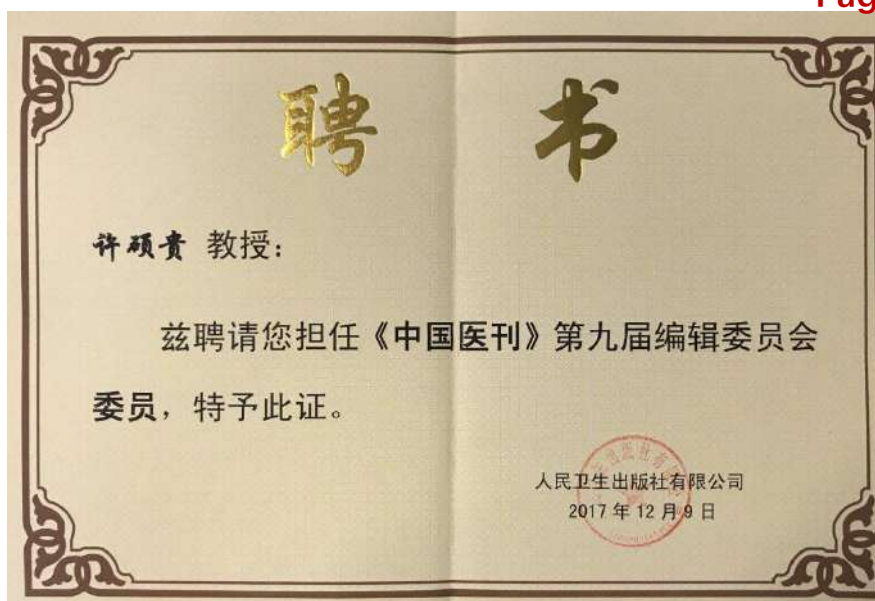
Shandong Medical Journal



Chinese Journal of Orthopaedic Trauma



Chinese Journal of Critical Care Medicine



Chinese Journal of Medicine

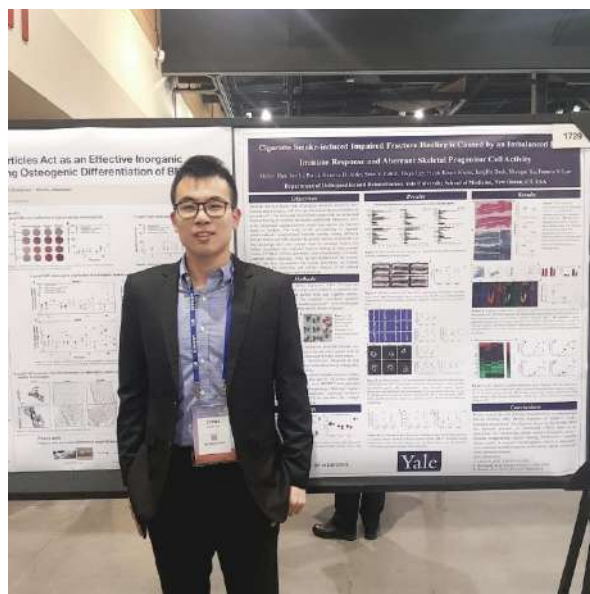
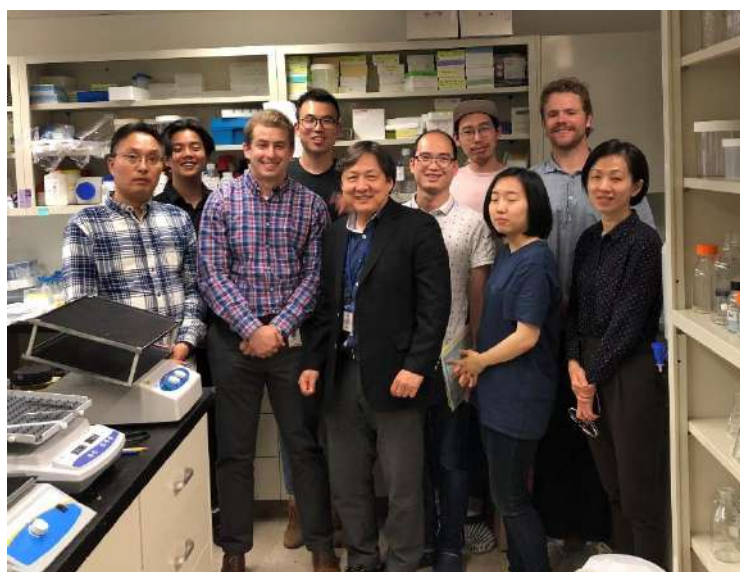


Chinese Journal of Emergency Medicine





Photos taken during the applicant's visits to the University of Louisville, Wake Forest University, and Stanford University School of Medicine in the United States



Photos taken during the applicant's doctoral student's joint training period at Yale University, and a photo of the two together

Allergy to Surgical Implants

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Abstract

Surgical implants are essential elements of repair procedures to correct worn or joint, damaged spinal components, heart and vascular disease, and chronic pain. However, many of the materials that provide stability, flexibility, and durability to the implants are also immunogenic. Fortunately, allergic responses to surgical implants are infrequent. When they do occur, however, the associated pain, swelling, inflammation, and decreased range of motion can significantly impair the implant function. Given the high numbers of joint replacements performed in the developed world, allergic reactions to orthopedic implants from the largest category of allergic response. The most important allergens in this category include nickel, cobalt, chromium, and bone cement. These allergens are also the most important in reactions to spinal surgery. Multiple cardiac and neurosurgical devices are considered of metals and adhesives that can be reacting to some individuals, implantable pacemaker generators, implants in cardiac pacemakers, pacemaker electrodes, and neurostimulators, may include components made of stainless steel, titanium alloy, platinum and iridium, epoxy resin, poly methyl methacrylates, and bioceramics, all of which are immunogenic in some patients. Cardiac stents and pacemakers are often made of Nitinol, a composite of nickel and titanium. Most surgical procedures are closed using skin glues, which are also capable of triggering a blushing contact dermatitis. Patch testing is the gold standard to determine sensitization, and this review provides a list of standard allergens to test for different implants. The patients most appropriate for testing include (1) preoperative joint replacement patients with a prior history of skin reactions to metal jewelry, joint wraps, mesh bands, metal glass lenses, artificial nails, or skin glue; (2) postoperative joint replacement failure patients needing revision without an obvious cause such as infection or mechanical incompatibility; and (3) postoperative cardiac or neurologic patients with localized rash, pain, swelling, or inflammation near or over the implant.

Keywords Surgical implant · Joint replacement · Joint fracture · Rash · Metal allergy · Nickel · Adhesive allergy · Methyl methacrylate · Contact dermatitis

Introduction

Surgical implants rely on a number of materials selected on the basis of their strength, stability, pliability, durability, and other physical characteristics. Unfortunately, some of these materials also have allergenic properties that can cause

immunological reactions and implant failure for a small number of patients. This review will provide background information on the number, kind, and revision rates of orthopedic joint implants, as these are the great majority of surgical implants used in the USA. They form the largest category of Medicare expenditures and, accordingly, revision due to allergy creates both a physical and financial burden on patient, surgeon, and payer. Implant components materials used, and repeated uses will illustrate the range of orthopedic implants and adverse outcomes reported. The materials and immune consequences of other implants, including cardiac pacemakers, pacemakers, and genetic and spinal stimulators, will be examined. While some surgeons express skepticism that allergy to orthopedic implants is a real condition, there is ample evidence in the published literature of both contact dermatitis and respiratory disease triggered by sensitization

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Determining the Mechanical Properties of Super-Elastic Nitinol Bone Staples Through an Integrated Experimental and Computational Calibration Approach

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Abstract

Super-elastic bone staples have emerged as a safe and effective alternative for internal fixation. Nevertheless, several biomechanical aspects of super-elastic staples are still unclear and require further exploration. Within this context, this study presents a combined experimental and computational approach to investigate the mechanical characteristics of super-elastic staples. Two commercially available staples with distinct geometry, characterized by two and four legs, respectively, were examined. Experimental four-point bending tests were conducted to evaluate staple performance in terms of generated forces. Subsequently, a finite element based calibration procedure was developed to capture the unique super-elastic behavior of the staple materials. Finally, a virtual bench testing framework was implemented to separate the effect of geometry from that of the material characteristics on the mechanical properties of the devices, including generated force, stress distribution, and fatigue behavior. The experimental tests indicated differences in the force vs. displacement curves between staples. The material calibration procedure revealed marked differences in the super-elastic properties of the materials employed in staple 1 and staple 2. The results obtained from the virtual bench testing framework have showed that both geometric features and material characteristics had a substantial impact on the mechanical properties of the device, especially on the generated force, whereas their effect on stress distribution and fatigue behavior was comparatively less pronounced. To conclude, this study advances the biomechanical understanding of Nitinol super-elastic staples by separately investigating the impact of geometry and material characteristics on the mechanical properties of two commercially available devices.

Keywords Bone staple · Orthopedic device · Nickel-titanium · Experimental testing · Digital twin · Material identification

Introduction

Bone staples are internal fixation devices that are commonly used in the foot, hand, and ankle to provide bone alignment and stabilization after fracture or bunion procedures [1, 17–21]. Nitinol bone staples have emerged as a safe and effective alternative for internal fixation [1, 15, 22, 23], exhibiting superior to stability, reduced surgery time and increased mechanical characteristics as compared to conventional

stainless steel staples [27, 28]. Upon insertion into predrilled holes, Nitinol staples overcome their initial shape and apply compressive forces at the deployment site, pulling fractured bones together to maintain proper alignment [14]. The shape recovery mechanism of these staples can be released either using super-elastic or to the shape memory properties characterizing Nitinol [16, 29]. Accordingly, Nitinol staples use dissimilar heat- or shape memory effects to activate shape recovery (or heat-activated staples). In the first type of Nitinol staples, austenite finish (A_f) transformation temperature of Nitinol [32] is approximately equal to or lower than room temperature. These staples must be held open by a surgical instrument before deployment. In the second type of Nitinol staples, the A_f transformation temperature of Nitinol is above room temperature. Thus, shape recovery is triggered by the thermal shape memory effect as the staple reaches body temperature or is heated to higher temperatures.

Alberto L. Audenino · Karin A. Pacheco received the review of this article.

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Research Article

Combustion Synthesis Porous Nitinol for Biomedical Applications

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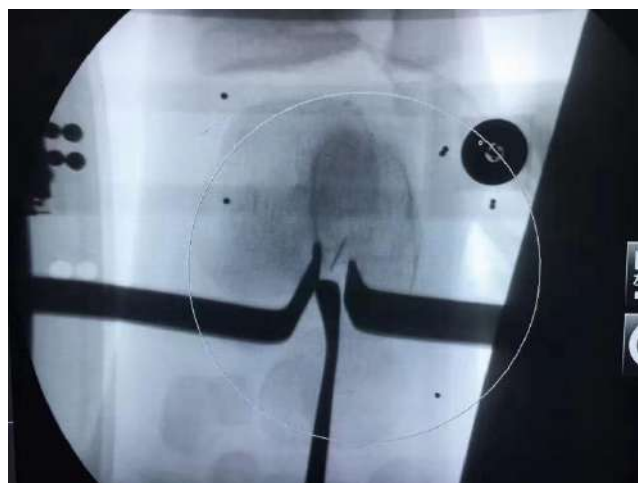
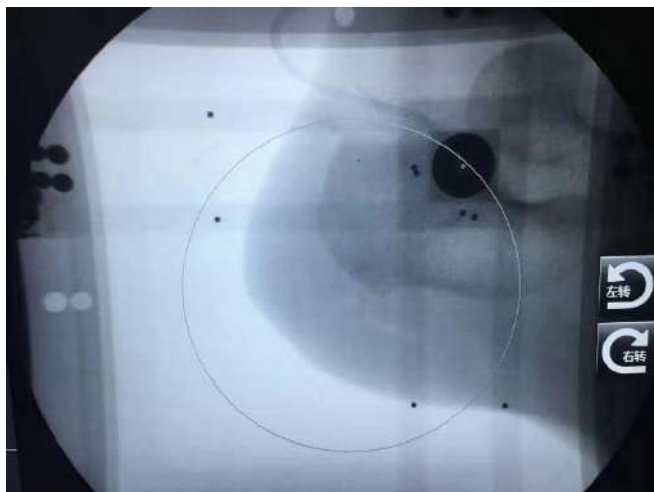
Porous Nitinol with a three-dimensional anisotropic, interconnected open pore structure has been successfully produced by the combustion synthesis (CS) of elemental Ni and Ti powders. The resulting product can be tailored to closely match the stiffness of cancellous bone to minimize stress shielding. The average elastic modulus was approximately 1 GPa for a porosity of 80 vol% and the average pore size of 100–200 μm . The low elastic modulus meets the basic demand for orthopedic bone ingrowth applications. Furthermore, porous Nitinol was composed of cubic (austenite) and martensitic (transformed) NiTi compounds, without the presence of Ni-rich or Ni-poor phases. The resulting product exhibits excellent corrosion resistance with breakdown potentials above 700 mV. An *in vivo* study in critical size of the tibia demonstrated rapid osseointegration into the porous structure as early as two weeks and complete bone growth across the implant at six weeks. A separate *in vivo* study showed complete through growth of bone at four months using a thicker interbody fusion model, substantiating the use of porous Nitinol as an implant material for applications in the spine. Porous NiTi is thus a promising biomaterial with proven biocompatibility and exceptional osseointegration performance which may enhance the healing process and promote long-term stabilization, making it a strong candidate for a wide range of orthopedic implant applications.

1. Introduction

Since the early 1960s, orthopedic implants were made from three distinct material types: metals, ceramics, and polymers. Commonly used materials are cobalt base metal alloys, stainless steel, titanium, polyetheretherketone (PEEK), zirconia, and alumina ceramics, poly lactic acid (PLA), or poly glycolic acid (PGA). Biodegradable materials, in spite, solid titanium has historically been used for spinal fusion implants. Since the early 2000s, PEEK was introduced and gained acceptance for its low modulus and radiolucency properties [1, 2]. However, recent studies have shown that PEEK does not integrate well with surrounding bone and may form a fibrous connective interface. Due to its hydrophobic and inert properties, the spine industry is undergoing a paradigm shift, moving back to metals to improve osseointegration at the bone-implant interface and enhance long-term implant stability [3].

Nitinol has widely been used for devices in cardiovascular, neurological and orthopedic devices. In its single state, Nitinol exhibits a modulus of elasticity of between 40–75 GPa depending upon its transformation temperature and crystalline texture. In contrast, wrought stainless steel and titanium exhibit moduli of up to 200 and 100 GPa, respectively [4], while the elastic modulus of cortical bone is between 15–20 GPa, that of cancellous is approximately 1–10 GPa [5–7]. The application of solid titanium or stainless steel devices can lead to stress shielding due to the mismatch of stiffness between the implant and the surrounding bone. “Stress shielding” refers to the bone healing and reduction of bone density that results when implants remove the stresses normally experienced by bone. Stress shielding is minimized by using implants that exhibit stiffness similar to the surrounding bone. Stiffness, in turn, is enhanced by both the design and by the elastic modulus of the implant material. The development of biocompatible materials that closely







Novel laser positioning navigation to aid puncture during percutaneous nephrolithotomy: a preliminary report

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Abstract

Purpose Accurate puncture of the renal collecting system is crucial to the success of percutaneous nephrolithotomy and presents a technical challenge for urologists. Here, we introduced the Surgical Approach Visualization and Navigation (SAVN) system, a novel navigation system to assist puncture and reduce intraoperative radiation.

Materials and methods Twenty kidneys of 10 cadavers were randomly divided into two groups for renal calyx puncture. In the control group, traditional fluoroscopy was used for guidance, while SAVN system was used in the experimental group. Puncture duration, number of puncture attempts, total number of intraoperative fluoroscopies, and number of fluoroscopies during the puncture procedure were recorded.

Results The puncture duration was 14.2 ± 2.5 s in SAVN group and 48.3 ± 7.1 s in conventional group ($P < 0.05$). One puncture attempt was needed for successful puncture in SAVN group, while more than one in conventional group ($P = 0.28$). The total number of intraoperative fluoroscopies was 3.3 ± 1.0 in SAVN group and 14.5 ± 3.1 in control group ($P < 0.05$), while the number of fluoroscopies during the puncture procedure was 0 and 11.2 ± 2.4 , respectively ($P < 0.05$).

Conclusions The novel SAVN system has a simplified structure and is easy to use. It can be used to successfully assist with puncture of the renal calyx, thus reducing puncture duration and radiation dose.

Keywords Kidney stones · Percutaneous nephrolithotomy · Puncture · Navigation

Introduction

Since its introduction in 1976, percutaneous nephrolithotomy (PCNL) has become the gold standard for the treatment of kidney stones larger than 20 mm in diameter [1], as it is minimally invasive and associated with few complications.

Jianghong Wu, Panyu Zhou and Xi Luo contributed equally to the article.

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